

Trauma Penetrating

Key Concepts

- Penetrating wounds are GSW, Stab, puncture or slash wounds to the head neck torso abdomen, axilla, buttocks or thigh area
- Survival is possible if bleeding can be surgically controlled
- The focus of care is bleeding control, rapid transport and supportive care
- Leave all impaled or penetrating objects in place as they may be limiting internal bleeding
- LOAD and GO limits on scene time as much as possible
- ACLS Medications have no role in Traumatic Arrest

MARCHES Protocol

- **Massive bleeding control** - tourniquet, wound packing
- **Airway** – NPA/OPA/Cricothyrotomy
- **Respiratory** – decompress chest if tension pneumothorax suspected, occlusive dressing for open pneumothoraces
- **Circulation**- IV/IO, pelvic binder, reassess tourniquet/wound packing
- **Hypothermia care**/Hike vs Helicopter (consider evac decision)
- **Eye injuries** – cover with rigid shield and no pressure on the eye
- **Spinal motion restriction** if indicated

EMT/AEMT

- Stop **Massive Bleeding** as first action
 - Tourniquet
 - Compress large vessel in the wound by hand against bone
 - Wound packing
 - Pressure dressing with firm direct pressure
 - Establish Simple **Airway** with NPA or OPA and suction secretions if needed
 - Support **Respirations**
 - BVM with Oxygen if needed
 - Support **Circulation**
 - Start CPR if in arrest
 - Reassess Tourniquets, bleeding wounds
 - Place on Monitor or AED
 - Prepare for transport or move patient towards Medic Unit
- **Continuously monitor tourniquet and wound packing interventions****
Reassess after every patient move

EMT/AEMT - Expanded Scope

- **TXA** per TXA Guideline

Paramedic

- Assess BLS **MARCHES** interventions
 - If in arrest or profound shock needle thoracostomy on affected side or bilateral if indicated
 - 4-5th intercostal space anterior axillary line (nipple level) above rib is preferred location
 - 2nd intercostal space, mid-clavicular is alternative location
- **Limit scene time** after BLS or lifesaving interventions performed
- **Airway**-If appropriate, consider advanced airway management or surgical cricothyrotomy
 - Capnography, if available, is required for all patients being ventilated
- **Respiration**- consider needle or open thoracostomy if unable to ventilate effectively or in arrest with thoracic trauma
- **Circulation**- Start IV/IO and administer fluids according to Trauma General Patient Care Guideline
- **Tranexamic acid (TXA)** as indicated per TXA Guideline
- Consider Trauma - Antibiotics Guideline